Behavioral health telehealth follow-up, depression and anxiety

A full doctor-patient conversation and the finished note it should produce, laid out in the Sully Behavioral Health Telehealth Note template. Use it to test an AI scribe, train new scribes, or see what a complete note captures.

Patient	31-year-old man (fictional)
Visit	Established patient, video visit, medication management
Length	About 12 minutes, 47 turns
Matching template	Behavioral Health Telehealth Note · sully.ai/templates/behavioral-health-telehealth-note

A Visit transcript

Clinician	Hi Marcus, can you hear me OK?
Patient	Yeah, you're a little frozen. Oh, there you go.
Clinician	Good. Before we start, can you confirm where you are right now? Just the city and state.
Patient	I'm at home. Columbus, Ohio.
Clinician	And are you somewhere private where you can talk freely?
Patient	Yeah, I'm in my car in the driveway actually. My roommate's home.
Clinician	That works. And if we get cut off, I'll call you back at the number ending in 4417. Is that still right?
Patient	Yep.
Clinician	Great. So it's been about six weeks since we bumped your sertraline up to 100. How have things been?
Patient	Better. Like, noticeably. Not perfect. But better.
Clinician	Tell me what better looks like.
Patient	I'm getting out of bed for work without the, like, forty-five minutes of staring at the ceiling. I went to my cousin's birthday thing, which a couple of months ago I would've bailed on. I'm cooking again a little.
Clinician	That's real progress. What's not perfect?
Patient	The anxiety's still there. Like, Sunday nights I get this dread about the work week. My chest gets tight. And I'm still not sleeping great. I fall asleep fine but I wake up at like 3:30 and my brain just starts going.

Clinician	What does your brain go to at 3:30?
Patient	Work stuff mostly. Did I send that email, is my manager annoyed with me. Money. Just, everything.
Clinician	How many nights a week?
Patient	Four, maybe five.
Clinician	Any side effects from the higher dose? Stomach upset, headaches, feeling jittery, any sexual side effects?
Patient	Stomach was weird the first week. That went away. The, um, the sexual thing, a little. It takes longer. It's not a deal-breaker.
Clinician	Thanks for telling me, a lot of people don't bring that up. If it becomes a problem, there are things we can do, so let me know. You filled out the PHQ-9 and GAD-7 before the visit. Your PHQ-9 was 9 today, down from 17 when we started. And the GAD-7 was 12, down from 15.
Patient	Huh. OK. That tracks.
Clinician	On the PHQ-9 you answered "not at all" for the last question, about thoughts of being better off dead or hurting yourself. I still like to ask directly. Any of those thoughts since we last talked?
Patient	No. Not this time. That was a dark stretch back in the spring, but no.
Clinician	I'm really glad. And the plan we made back then, you still have the 988 number saved?
Patient	Yeah, it's in my phone.
Clinician	Good. How about alcohol? Last time we talked about cutting back.
Patient	I'm down to weekends. Like, three or four beers on a Saturday. I stopped drinking alone on weeknights.
Clinician	That's a big change. Has that affected sleep at all?
Patient	Weekends I sleep worse, actually. Now that you say that.
Clinician	Alcohol tends to cause that early-morning waking. Something to notice. Are you still seeing the therapist?
Patient	I've done four sessions with her. She's doing the, um, CBT stuff. Writing down the thoughts. It's helping with the Sunday thing a bit.
Clinician	Good. So here's what I'm thinking. The depression is clearly responding, and I don't want to rock that. The anxiety and the sleep are lagging behind a bit, which is common, anxiety often takes longer. Sertraline can still go up, but you're six weeks in at 100 and still improving, and the side effect is noticeable. So I'd hold at 100 for another six weeks rather than push it now.
Patient	OK. I'd rather not make it worse, the, you know.
Clinician	Right. For sleep, I'd like to try something non-medication first. There's a specific therapy for insomnia called CBT-I. Your therapist may already do it, so ask her. In the meantime: if you're awake more than about twenty minutes at 3:30, get up, go to another room, do something boring in dim light, and go back when you're sleepy. Same wake time every day, weekends too.
Patient	Even weekends?
Clinician	Even weekends. That's the annoying part. And keep the phone out of the bed. If we try that for a month and it's not budging, we can talk about a medication.

Patient	OK. I can try.
Clinician	Also, for the Sunday chest tightness, did your therapist teach you the paced breathing?
Patient	She did. I forget to do it.
Clinician	Set an alarm for Sunday at 7 pm. Literally a reminder that says "breathe." It sounds silly, but it works.
Patient	*(laughs)* OK.
Clinician	I'll send the sertraline 100 refill to the same pharmacy, the CVS on High Street?
Patient	Yes.
Clinician	And let's meet again in six weeks, video again. If anything changes before then, especially any thoughts of hurting yourself, call 988 or our office, or go to the ER.
Patient	Will do. Thanks.
Clinician	Take care, Marcus. Nice work these past weeks.

B Finished note

Visit: Synchronous audio-video telehealth. Established patient. Medication management follow-up.

Telehealth elements: Patient location verified: home (vehicle in driveway), Columbus, Ohio. Clinician licensed in patient's state. Patient confirmed private setting. Backup phone number confirmed. Consent to telehealth on file.

Patient: Marcus Hill (fictional), 31 M.

Subjective: Six weeks after sertraline increase to 100 mg daily. Reports improved mood, energy and motivation: getting up for work without prolonged delay, attending social events, cooking again. Persistent anticipatory anxiety Sunday evenings with chest tightness. Sleep: normal onset, early-morning awakening ~3:30 am with ruminative worry (work, finances), 4 to 5 nights/week; worse on weekends after alcohol. Side effects: transient GI upset (resolved), mild delayed orgasm, tolerable. Alcohol reduced to weekends only, 3 to 4 beers on Saturday; no longer drinking alone on weeknights. Engaged in CBT with therapist, 4 sessions completed.

Measures: PHQ-9: 9 (baseline 17). GAD-7: 12 (baseline 15).

Risk assessment: Denies current suicidal ideation, intent or plan (PHQ-9 item 9 = 0; asked directly). History of passive ideation in spring, resolved. Safety plan reviewed; 988 saved in phone. Protective factors: engaged in treatment, social connection, employment. Risk judged low.

Mental status (via video): Casually dressed, good eye contact. Speech normal rate and tone. Mood "better." Affect mildly anxious, reactive, appropriate. Thought process linear. No SI/HI. No perceptual disturbances. Insight and judgment good.

Assessment:

1. Major depressive disorder, single episode, moderate, improving on sertraline.
2. Generalized anxiety disorder, partially responding.
3. Insomnia with early-morning awakening, likely related to anxiety and alcohol.
4. Alcohol use, reduced; continue monitoring.

Plan:

- Continue sertraline 100 mg daily; hold dose for 6 weeks given ongoing improvement and mild sexual side effect. Refill sent to pharmacy on file.

- Insomnia: CBT-I principles reviewed (stimulus control, fixed wake time, phone out of bed); ask therapist about CBT-I. Consider medication if no improvement in 4 weeks.
- Paced breathing with Sunday evening reminder.
- Continue CBT with therapist.
- Counseled on alcohol and sleep.
- Crisis plan: 988, office, or ED.
- Follow-up: 6 weeks, video.

Time: 12 minutes.

How to use this sample. Run the transcript through your scribe and compare its draft with the finished note. Check the details that are easy to miss: doses, which side of the body, what the patient said no to, and every follow-up step. For 536 more transcripts, many with audio and reference notes, see github.com/aniquethaplawala/open-clinical-encounters.

Rather not write the note yourself? Sully's AI Scribe listens to the visit and drafts the note in your template, ready for you to review and sign.

sully.ai

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